National Cancer Control Programme (NCCP) 00563 · clinician draft

Encorafenib and binimetinib for advanced BRAF-mutated melanoma

Your diagnosis and treatment setting NCCP 00563: adults with advanced unresectable or metastatic melanoma with a BRAF V600 mutation.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Continuous oral treatment

Active medicines: Encorafenib + Binimetinib

Emetogenic risk: Moderate to high (oral)

Medicine	Dose / route	When
Individual anti-sickness / rescue plan	To be completed by the prescribing team if needed	As specified in the individual plan; no routine schedule issued here Source emetogenic category and local policy guide the decision. No medicine, dose or administration order is selected by this draft.

Additional phase-specific support

- Assess tumour lysis risk when tumour burden, kidney problems or dehydration raise concern; fluids and additional tests are individual prescriptions. Both oral components are moderate-to-high emetic risk in the source.
- Use effective contraception during treatment and for at least one month after the last dose. Hormonal contraception needs an additional or alternative barrier method because interactions can reduce effectiveness. Discuss fertility beforehand and follow the team's breastfeeding instructions. Report a possible pregnancy promptly.
- Contact the team promptly for new visual symptoms, chest pain, breathlessness, palpitations, faintness, severe muscle pain or weakness, dark urine, bleeding, severe rash, persistent diarrhoea or severe abdominal pain. Do not continue one medicine at an unchanged dose if the other has been held without a specific team instruction. Report markedly reduced urine, persistent vomiting, new muscle cramps or weakness, or palpitations promptly; the team may need to check salts, kidney function and tumour lysis risk.
- This is not an issued prescription. The clinician and pharmacist must complete and authorise any supportive medicines.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00563 v4 · p8–10 · NCCP regimen and local supportive-care policy

Sources checked 2026-11-06 · SACTCheck v0.84.40 · Regimen-level drafts; prescriber and oncology-pharmacy verification required